



جوستاف روسي انترناشيونال - مصر (EPE) * طلب علاج أول مرة / متردد

إسم المريض ربيعاً : سحر شعبان قرني محمد
المحافظة : القاهرة
الحى / المدينة : حلوان
العنوان : ممم
رقم البطاقة : 28205030102424 رقم التليفون : 01000476510
العمر (تاريخ الميلاد) : 03-05-1982 المهنة : ربه منزل
تاريخ تسجيل الإستمارة : 01-10-2026

الإقرار

أقر انا الموقع أدناه انني لا اتمتع باي نظام علاج مثل :-التأمين الصحي-القوات المسلحة-الشرطة-الهيئات-المؤسسات او اي نظام يتعارض مع نظام العلاج علي نفقة الدولة- وسوف اتحمل كافة نفقات العلاج في حالة ثبوت غير ذلك
الإحصائي الاجتماعي
التوقيع: (-----)
المقرر بما فيه
التوقيع: (-----)

التشخيص الإكلينيكي : سرطان بالثدي (مبادرة)
نتائج الأبحاث : (مرفق)
التشخيص النهائي : سرطان بالثدي (مبادرة)
خطة العلاج : علاج مبادرة سرطان الثدي علاج تدعيمي وتلطيفي [6579] قسم العلاج : الاورام
التعليق على التشخيص : -----

تقرير اللجنة الثلاثية المتخصصة

الاسم : د/محمد كيلاني
الاسم : د/ندى حسن
الاسم : د/نرمين رمضان
الوظيفة :
الوظيفة :
الوظيفة :
التوقيع :
التوقيع :
التوقيع :
أطلعت اللجنة علي لائحة العلاج الخاصة بالمجالس وتلتزم جهة العلاج بلائحة المجالس الطبية. كما تقرر اللجنة بان البيانات المدونة فى هذا النموذج صحيحة.
تعتمد صحة البيانات المرفقة
رئيس اللجنة ومدير
المستشفى.

تتعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض وإعادة مناظرة المريض بالمستشفى قبل الصرف.

أعضاء اللجنة

د. / د. / د. / د. / د. / د. / د. / د. / د. / د.

قرار اللجنة

التشخيص النهائي :
خطة العلاج :
المستشفى :
قسم :
جهة العلاج :
نفقات :
ملاحظات :

ختم
شعار الجمهورية

مدير عام
مديرية الشؤون الصحية

رئيس
المجلس الطبي المتخصص

--

Name: سحر شعبان قرني محمد National ID: 28205030102424	
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تقرير طبي

السيد
تحية طبية وبعد,,

إيماء للطلب المقدم من	بشأن تقرير طبي للمريض
بقسم:	رقم طبي:

نتشرف بأن نوضح فيما يلي تقريراً عن حالته
--

تاريخ وساعة الدخول : -- --:-- yyyy-mm-dd	تاريخ وساعة الخروج : PM 01:17 05-07-2026
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وبعد الكشف والفحوصات والمتابعة وجد أنه
--

المریضة تعاني من ورم غير حميد بالثدي و تحتاج الي Supportive and palliative Treatment
--

التاريخ : 05-07-2026	الوقت : -- --:--
	الطبيب المعالج : د/شيماء لاشين



Dms.
Cassini



Name: سحر شعبان قرني محمد			
MR. : 2095			
Detailed Medical Report			
		السيد	
تحتية طبية وبعد,,			
إيماء للطلب المقدم من		بشأن تقرير طبي للمريض	
بقسم:		رقم طبي:	
ننشرف بأن نوضح فيما يلي تقريراً عن حالته			
تاريخ وساعة الدخول :		تاريخ وساعة الخروج :	
وبعد الكشف والفحوصات والمتابعة وجد انه			
The patient is a 43-year-old with a newly diagnosed, premenopausal right-sided breast cancer, clinical stage T2 N0 M0, estrogen receptor (ER) 8/8 positive, progesterone receptor (PR) 4/8 positive, HER2 SISH amplified. Neoadjuvant therapy was initiated with a Taxotere 130 mg and Carboplatin AUC 5 regimen, with planned completion by cycle 5 and transition to PHESGO on 5 February 2026. An echocardiogram on 1 May 2026 showed an ejection fraction of 63 %. On examination on 1 May 2026 a right-sided liquid mass measuring 2 × 2 cm was noted on ultrasound, and surgical consultation was requested. Post-operative planning for PHESGO was pending, with a multidisciplinary team decision anticipated after cycle 8. An echocardiogram performed on 17 February showed an ejection fraction of 65 %. The patient was scheduled for further evaluation on 19 March 2026. The patient received neoadjuvant chemotherapy consisting of TCHP (docetaxel, carboplatin, trastuzumab, pertuzumab) followed by right-sided breast-conserving surgery (BCS) for an invasive ductal carcinoma, grade 2, T2 N0. Post-operative management included placement of a port for trastuzumab-deruxtecan (TDM-1) therapy for up to one year, together with adjuvant endocrine therapy (Femara and Zoladex). Adjuvant therapy was initiated in February 2026 after right-sided BCS (pathology T2 N2 M0). The plan included port-mediated TDM-1, Femara, and Zoladex. The first cycle of TDM-1 (dose 226 mg) was administered, with baseline labs acceptable; weight 63 kg, height 162 cm, and echocardiogram on 17 February showing EF 65 %. During cycle 2, the patient received Kadcylla 226 mg together with Femara. Laboratory parameters remained within acceptable limits. In cycle 3, Kadcylla 225 mg was administered along with Femara; a supplemental echocardiogram on 17 February confirmed EF 65 %, and labs drawn on 23 May were acceptable. The patient underwent right-sided BCS on 23 February 2026 (pathology ypT2 ypN0). Zoladex and Femara were continued from April 2026, and a port was placed in anticipation of cycle 4 Kadcylla 250 mg when the drug became available. On examination on 19 July 2026, the right breast showed thickening consistent with fibrosis on ultrasound, with an ejection fraction of 65 %. Cycle 5 Kadcylla 220 mg (weight 60 kg) was administered together with Femara. On 10 September 2026, the patient received cycle 7 Kadcylla 220 mg and continued Femara; calcium, vitamin D, and methyl-techno supplementation were noted, with grade 1 peripheral neuropathy and Zoladex scheduled to be due on 19 September. The patient has now completed seven cycles of			
تحريراً في ,			
التاريخ :		18-09-2026	
الوقت :			
الطبيب المعالج :		dina makram	
يعتمد ,			

03/24/2026

MDT FORM

Name: Sahar Shaban Orah Mohamed	MRN: 2095
Age: 44 Y	ID: 28205030102424
Menopausi Status:	Phone: 01000476510
NRS SCORE:	Marital Status: MARRIED
BMI:	Family History: -VE FH
CO Morbidity: NDT DM OR HTN	Reason Of MDT: FOR ADJUVANT TTT

Clinical Data:

BX: IDC G2
ER 8/8 **PR** 4/8 **HER** +3 B\ **KI67**

Sonomammography:

Mets Workup:

History:

43 Y PREMENOP RT BREAST CANCER CT2 NO MO
ER8/8 PR4/8 HER2 SISH AMPLIFIED CLIPS INSERTED
MULTIFOCAL SA 1.67

PT RECEIVED 6 CYCLE OF TAXOTERE +
CARBOPLATIN AUC 5 NEOADJ AND RECEIVED 7

Recommendations:

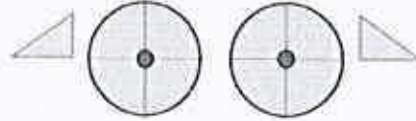
FOR PORT
FOR TDM1
FOR AJ + zoladex

MDT Outcome & Plan

FOR PORT
FOR TDM1 up to 1 year
FOR Femara + zoladex

R

L



T N M

SIGNATURE

MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY	Nutrition	Psychiatric
د/ هبة عبد العزيز	د/ محمد الشاذلي				

Patient Name : الأستاذ / سحر شعبان قرني محمد	Request No. : 13005159
Patient No. : 28205030102424	Request Date : 23/02/2026
Path No. : 512/26	Verification Date : 28/02/2026
Sex : Female	Financial Category : هوسلاف روسي - نفقة التولية
Age : 43 Y	Department : Out Patient
Referring Doc :	Registered by : Mrs. Mayada Hegazy

Pathology Report

Clinical Data: Right breast cancer, Post-NACT.
Nature of the specimen: Right breast conservative surgery, Sentinel lymph nodes biopsy & Extra-lymph node.

Gross Examination:

- Right breast conservative surgery specimen measured 8x5x3 cm, with covering skin ellipse measured 5x2 cm.
- Serialing showed a wire guided tumor bed site measured 4x1x1 cm, located 0.3 cm away from the least lateral margin.
- Sentinel lymph nodes: Multiple fibrofatty tissue pieces collectively measured 4x4 cm, entangling 5 lymph nodes, the largest measured 1.5x1x0.5 cm.
- Extra-lymph node: Single nodule measured 1x1x0.5 cm.
- 17 Blocks:
- 1- Medial margin., 2- Lateral margin., 3- Superior margin., 4- Inferior margin., 5- Deep margin., 6- SLNs., 7-16- TBS., 17- Extra-lymph node.
- Gross was done by: Dr. Heba El-Shafeey.

** Frozen (Intra-operative) Results:

- Free surgical margins.
- Five free sentinel lymph nodes, SLNs (0/5).

Microscopic examination:

- Histological type: Residual viable invasive duct carcinoma with wide micropapillary features (Actual tumour size 4x1 cm) (Cellularity 20%).
- Grade 2 (differentiation 3, nuclear pleomorphism 2, mitosis 2).
- 25% Intermediate grade ductal carcinoma in situ (Solid & cribriform pattern with comedo type necrosis).
- Angioinvasion: Present.

Verified by :

Dr. Duaa Mohamed

Printing Date : 03/03/2026 11:24:10

Page 1 of 2

Printing By : marwa mohamed youss



Patient Name : الأستاذ / سحر شعبان فرني محمد
Patient No. : 28205030102424
Path No. : 512/26
Sex : Female
Age : 43 Y
Referring Doc :

Request No. : 13005159
Request Date : 23/02/2026
Verification Date : 28/02/2026
Financial Category : جوسثاف روسي - نفقة الدولة
Department : Out Patient
Registered by : Mrs.Mayada Hegazy

Pathology Report

- Perineural invasion: Absent.
- Microcalcification: Present.
- Skin: Free.
- Surgical margins: Free.
- Sentinel lymph nodes: Five lymph nodes free of metastatic deposits, (0/5).
- Extra-lymph node: One free lymph node (0/1).
- Total lymph nodal status: Six lymph nodes free of metastatic deposits (0/6).
- Histologic response to therapy: Pathologic partial response to therapy.

Diagnosis:

Right breast cancer (Post-NACT), Right breast conservative surgery, Sentinel lymph nodes biopsy & Extra-lymph node biopsy:

Residual viable invasive duct carcinoma with wide micropapillary features, Grade 2 (Cellularity 20%) (Actual tumor size 4x1 cm).

- 25% Intermediate grade ductal carcinoma in situ (Solid & cribriform pattern with comedo type necrosis).

- Skin: Free.
- Surgical margins: Free.
- Sentinel & Extra-lymph nodes: Six lymph nodes free of metastatic deposits (0/6).
- Stage: ypT2, ypN0 (sln).
- RCB: 1.687, RCB class-II.

Dr. Walaa Ibrahim Ghoneim.
Dr. Heba El-Shafeey.

Signature

Verified by :
Dr Duaa Mohamed

Printing Date : 03/03/2026 11:24:10

Page 2 of 2

Printing By : marwa mohamed youss



غرفة رقم (4) - الدور الأول - مبنى رقم (987) - كورنيش النيل - مصر القديمة - القاهرة - مصر
Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qadema - Cairo, Egypt

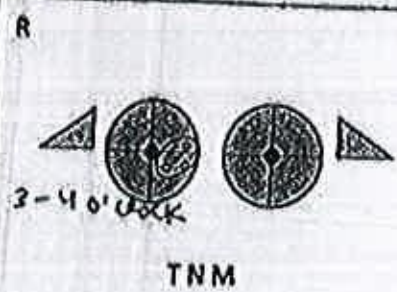
MDT FORM

6/10/2025 G

Name	م. ش. ع. ن.	MRN	1448-5050
Age	42	ID	6460444444
Menopausal status	Pre	Phone	01111111111
NRS SCORE		Marital status	
BMI		Family history	
Comorbidity	Free	REASON OF MDT	

11/10/2025

Clinical Data			
BX:	IOC GIL		
ER:	8/8	PR:	7/8
HER2:	3+	KI67:	35%
Sonomammography:	multiple BI-RADS 3-4		
Mets Workup:	CT CAP free		



Recommendations:

For primarily 5 & Delip insertion
1- CSM
2) Neo Adj. CT
3) Rt BCS & SNLB

MDT OUTCOME & PLAN

For Dr. M. Sh. E. N. the stat NAC
(TCHP X5) guided by clinical response
the results of the biopsy & the results of the RT-PCR

SIGNATURE					
MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY	Nutrition	Psychiatric
[Signature]	[Signature]	[Signature]			

MDT FORM

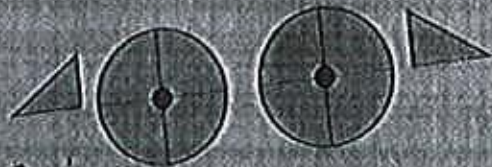
27/1/2026 G

Name	ح. م. ف. م.	MRN	2095
Age	43y	ID	2498/2026
Menopausal status	pre-0	Phone	01000 476510
NRS SCORE		Marital status	CA 100.1.0505
BMI		Family history	
CO morbidity		REASON OF MDT	

Clinical Data

BX:	INC II
ER:	4/8
PR:	4/8
HER2:	2+ STN. Aug 10
KI67:	

R PS: 1



Sonomammography:

Preop Rt Breast ca

Mets Workup:

claps CT2. No. 10
used mally

Rec. MACT
6 Taxol, Colchicine

TNM

Recommendations:

Sphero
with good Regimen Case

for RT BCS & SLNB
on wine

MDT OUTCOME & PLAN

RT BCS & SLNB
on wine

SIGNATURE

MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY	Nutrition	Psychiatr
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Patient Name : الأستاذ / دكتور شهاب لرامي محمد
Patient No. : 28205030102424
Path No. : SR-1215/25
Sex : Female
Age : 43 Y
Referring Doc :

Request No. : 13006627
Request Date : 03/09/2025
Verification Date : 09/09/2025
Financial Category : جوماتك روسي - فئة الدولة
Department : Out Patient
Registered by : Mrs. Mayada Hegazy

Pathology Report

- Ki67%: 35%.

* Scoring system: "ER, PR"

ASCO-CAP ER/PR Test Guideline Recommendations: proportion of cells staining (0, no staining; 1, >1%; 2, between 1% and 10%; 3, between 11% and 33%; 4, between 34% and 66% and 5, between 67%-100% of the cells staining). Intensity of positive tumor cells (0, none; 1, weak, 2, Intermediate; and 3, strong). Total score from 0 to 8.

* Scoring system: "Her2"

Reference: ASCO-CAP Her2 Test Guideline Recommendations, 2013. IHC 0: as defined by no staining observed or membrane staining that is incomplete and is faint/barely perceptible and within < 10% of the invasive tumor cells. IHC 1+: as defined by incomplete membrane staining that is faint/barely perceptible and within > 10% of the invasive tumor cells. IHC 2+: based on circumferential membrane staining that is incomplete and/or weak/moderate and within >10% of the invasive tumor cells or complete and circumferential membrane staining that is intense and within < 10% of the invasive tumor cells. IHC 3+: based on circumferential membrane staining that is complete, Intense > 10% of tumor cells.

Signature
Dr. Walaa Ibrahim Ghoneim
Dr. Heba El-Shafeey.

Verified by :
Dr. Duaa Mohamed

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Page 2 of 2

Printing By : Mrs. Mayada Hegazy

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Patient Name : الأستاذ / منير شعبان لطفى محمد
Patient No. : 28205030102424
Path No. : SR-1215/25
Sex : Female
Age : 43 Y
Referring Doc :

Request No. : 13006627
Request Date : 03/09/2025
Verification Date : 09/09/2025
Financial Category : جرمات روسي - ملنة الدولة
Department : Out Patient
Registered by : Mrs. Mayada Hegazy

Pathology Report

Clinical Data: Right breast mass.

Nature of Specimen: Referred block & slides (Tru-cut biopsy).

Gross Examination:

One referred block labelled by (634-2025) & patient name. Together with, five referred slides including 1 H&E stained slide and 4 Immunostained slides for (ER, PR, HER2neu & Ki67).

Microscopic Examination:

- Histological type: Invasive duct carcinoma.
- Grade 2 (differentiation 2, nuclear pleomorphism 2, mitosis 2).
- 5% Low grade ductal carcinoma Insitu (Solid & cribriform patterns).
- Angiolymphatic Invasion: Absent.
- Perineural Invasion: Absent.

* IPT:

- ER: Strong nuclear reaction in 95% of tumor cells (score 8/8).
- PR: Strong nuclear reaction in 60% of tumor cells (score 7/8).
- Her2: Strong complete membrane reaction in more than 10% of tumor cells (score 3+).
- Ki67: 35%.

Diagnosis:

Right breast mass, Referred block & slides (Tru-cut biopsy):
- Invasive duct carcinoma, Grade 2, with 5% Low grade ductal carcinoma Insitu (Solid & cribriform patterns).

IPT:

- ER: Positive (score 8/8).
- PR: Positive (score 7/8).
- Her2: Positive (score 3+).

Verified by :

Dr. Duaa Mohamed

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Page 1 of 2

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Room No. (4) - First Floor - Building No. 987 - Nile Corniche - Nisar El Qadima - Cairo, Egypt



ECHO REPORT

Name: سید محمد علی قرنی

Date: 29 / 9 / 2025

EF	68%	55-75%	PWT	0.8	0.6-1.1cm
LV EDD	4.5	3.5-5.8cm	AO.R	2.3	2-3.7cm
LVESD	2.1	2-4cm	LA	3.6	2-4cm
FS	30%	25-45%	RV		0.7-2.3cm(supine)
SWT	0.3	0.6-1.1cm	AO.CUSP		1.5-2.6cm

LV dimensions Normal

LV wall thickness Normal

LV diastolic function Normal

SWMA No R-wMA

LA and Ao root dimensions Normal

Mitral Valve

Aortic valve N

Tricuspid valve N

Pulmonary valve

Right side and systolic function APSE: 2.5

Intracardiac masses and effusion

Dr / محمد علی قرنی

أ.د. ناجية عبد الوهاب

تاريخ الفحص : 14/8/2025
كود المريض : 112332
رقم العيادة : 11250812162
Thank you for referring this case,

اسم : أستاذ الأشعة - جامعة القاهرة
سحر شعبان قرني
السن : 42
جهة التحويل :
الطبيب المعالج أ.د. : اسلام عبدالرحمن

High Resolution Ultrasound Examination Of Both Breasts Revealed:

- Homogeneous fibro-glandular breast parenchyma.

Right Breast:

- Multiple ill-defined spiculated heterogeneously hypoechoic masses surrounded by echogenic halo of desmoplastic reaction are noted in the LIQ. These are located at 3-4O'clock, 3.5mm deep to the skin. The larger mass is seen at 4O'clock 20mm from the nipple measuring about 30x20mm.
- Smaller masses are seen as follows:
- At 4O'clock adjacent to the index mass measuring about 10x8mm
- The farthest from the index mass is seen distal from the it measuring about 25x13mm
- Normal skin thickness and nipple-areola complex
- No pathologically enlarged level I, II & III axillary-lymph nodes or supraclavicular LNs

Left Breast:

- No breast solid or cystic lesions.
- Normal central ducts system.
- Normal skin thickness and contour as well as nipple areola complex.
- Nonspecific axillary LNs

Opinion:

- Multifocal right breast malignant mass for histopathological confirmation (BIRADS 5)
 - Normal study of left breast (BIRADS 1)
- NNB: Mammography is highly recommended for farther assessment

Best regards,

Dr. Amira H. Radwan M.D
Breast Imaging Consultant,
NCA, Cairo University

14/8/2025

Patient's Name: Sahar Shaaban Qurni Mohammed
Date: 2026-08-12
Referred by: Prof.Dr. no doctor
Hospital: Goustave Roussey International Egypt



Bilateral Digital soft tissue mammography with high resolution breast sonography revealed:

Clinical summary: Status post right CBS routine follow up study.

Findings:

Right breast.

- Scattered fibroglandular densities. (ACR b)
- Thickened skin and coarsen trabeculae.
- LIO focal architectural distortion
- no speculations, masses, asymmetry, or suspicious microcalcifications.
- **Ultrasound** demonstrated LIO focal area of parenchymal disruption underneath the operative scar with no solid lesions or fluid collections.
- Thickened skin and coarsen trabeculae.
- Clear right axilla. No pathological infra or supra-clavicular lymph nodes

Left breast.

- Scattered fibroglandular densities. (ACR b)
- No speculations, masses, asymmetry, distortion or suspicious microcalcifications.
- **Ultrasound** demonstrated no solid or cystic lesions.
- Normal skin, nipple/areola.
- Non-specific axillary LNs.

Conclusion: -

- **Right breast (BIRADS 2): Post-Therapeutic sequelae.**
- **Left breast (BIRADS 1): Normal study**

Recommendation: -

- **For annual follow up.**

Best Regards,
DR. Marwa M.Hassan



Verified by :
Dr. Duaa Mohamed

- Microscopic examination:
- Histological type: Residual viable invasive duct carcinoma with wide micropapillary features (Actual tumour size 4x1 cm) (Cellularity 20%).
 - Grade 2 (differentiation 3, nuclear pleomorphism 2, mitosis 2).
 - 25% Intermediate grade ductal carcinoma in situ (Solid & cribriform pattern with comedo type necrosis).
 - Angiolymphatic invasion: Present.

** Frozen (Intra-operative) Results:

- Free surgical margins.
- Five free sentinel lymph nodes, SLNs (0/5).

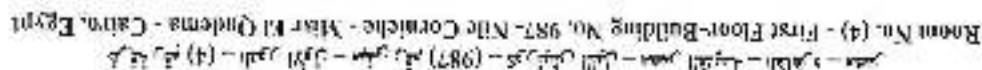
- Gross Examination:
- Right breast conservative surgery specimen measured 8x5x3 cm, with covering skin ellipse measured 5x2 cm.
 - Serialing showed a wire guided tumor bed site measured 4x1x1 cm, located 0.3 cm away from the least lateral margin.
 - Sentinel lymph nodes: Multiple fibrotically tissue pieces collectively measured 4x4 cm, entangling 5 lymph nodes, the largest measured 1.5x1x0.5 cm.
 - Extra-lymph node: Single nodule measured 1x1x0.5 cm.
 - 17 Blocks:
 - 1- Medial margin., 2- Lateral margin., 3- Superior margin., 4- Inferior margin., 5- Deep margin., 6- SLNs., 7-16- TBS., 17- Extra-lymph node.
 - Gross was done by: Dr. Hiba El-Shafey.

Clinical Data: Right breast cancer, Post-NACT.
Nature of the specimen: Right breast conservative surgery, Sentinel lymph nodes biopsy & Extra-lymph node.

Pathology Report

Path No.	512/26	Request No.	13005159
Sex	Female	Request Date	23/02/2026
Age	43 Y	Verification Date	28/02/2026
Referring Doc	Dr. Duaa Mohamed	Department	Out Patient
Path No.	512/26	Registered by	Mrs. Mayada Hegazy
Sex	Female	Financial Category	Dr. Duaa Mohamed
Age	43 Y	Request No.	13005159
Referring Doc	Dr. Duaa Mohamed	Request Date	23/02/2026
Path No.	512/26	Verification Date	28/02/2026
Sex	Female	Department	Out Patient
Age	43 Y	Registered by	Mrs. Mayada Hegazy

Dr. Duaa Mohamed



Printed by : Mawla Johanned Youssef

Dr. Walea Ibrahim Ghoneim,
Dr. Heba El-Shatey.

Signature

- Perineural invasion: Absent.
 • Microcalcification: Present.
 • Skin: Free.
 • Surgical margins: Free.
 • Sentinel lymph nodes: Five lymph nodes free of metastatic deposits, (0/5).
 • Extra-lymph node: One free lymph node (0/1).
 • Total lymph node status: Six lymph nodes free of metastatic deposits (0/6).
 • Histologic response to therapy: Pathologic partial response to therapy.
- Diagnosis:**
 Right breast cancer (Post-NACT), Right breast conservative surgery, Sentinel lymph nodes biopsy & Extra-lymph node biopsy;
 Residual viable invasive duct carcinoma with wide micropapillary features, Grade 2 (Cellularity 20%) (Actual tumor size 4x1 cm).
 • 25% Intermediate grade ductal carcinoma in situ (Solid & cribriform pattern with comedo type necrosis).
 • Skin: Free.
 • Surgical margins: Free.
 • Sentinel & Extra-lymph nodes: Six lymph nodes free of metastatic deposits (0/6).
 • Stage: ypT2, ypN0 (sln).
 • RCB: 1.687, RCB class-II.

Pathology Report

Patient Name : _____ / _____ / _____ (N/A)

Patient No. : 28205030102424

Birth No. : 512/26

Sex : Female

Age: 43 Y

Referencing Doc :

Request No. : 13005159

Request Date : 23/02/2026

Verification Date : 28/02/2026

Financial Category:

Department : Out Patient

Registered by : Mrs. Mayada Hegazy